Analysis of a Health System Specialty Pharmacy Model

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ABSTRACT

In the ever-growing field of Health System Specialty Pharmacy, there is a need for benchmarking and understanding productivity measures. Typical pharmacy metrics focus on the number of prescriptions dispensed and allocate a specific amount of time to each dispense.[1] A productivity tool was developed based on median times from pharmacist and technician responses to the average times spent completing different tasks. These tasks were then input into a dashboard to track the total amount of time spent by individual staff completing each task as well as time spent being productive. Results from this benchmarking and productivity tool have provided a foundation for evaluation of productivity in clinical, operations, and shipping in a Health System Specialty Pharmacy. To further improve the model, data will need to continue to be evaluated over a longer period of time and the benchmark used to project future needs for staffing based on data trends, in order to utilize the tool to justify staffing additions.

BACKGROUND

- The current productivity model used focuses on the number of prescriptions dispensed and correlates hours to that dispense.
- Utilizing a model based solely on prescription volume fails to capture productivity outside of dispensing.
- The American Society of Health Systems Pharmacist's Section of Specialty Pharmacy Practitioners published a white paper to develop internal benchmarking for health systems that identified key performance indicator development from four functional areas, including: pharmacy operations, pharmacist consulting services, prior authorization services, and medication assistance.[2]
- A specialty pharmacy productivity tool that represents the team-specific tasks involved in specialty pharmacy workflow may be more reflective of specialty pharmacy productivity compared to a flat weight scoring system for new vs. refill prescriptions.[1]

OBJECTIVE

- Objective: To develop and evaluate an internal set of criteria that measures specialty pharmacy productivity of pharmacists and technicians.

METHODS

- A productivity tool was developed to track time spent in defined productivity tasks based on recorded "wrap up" tags in the health system electronic medical record.
- Staff were surveyed to assign the average amount of time spent on completing each task for a single patient and asked what tasks were not listed.
- A total of 21 unique survey responses were collected.
- Tasks that staff input as not being listed were used to create a second survey. The survey was distributed to staff to assign the average time spent on completing these tasks.
- "Wrap up" tags were each paired with the median time spent in minutes on that task.
- Data on "wrap up" tags utilized by staff members was pulled from January 1, 2022 – April 30, 2023.
- Data was categorized into 3 areas: Clinical Hours, Shipping Hours, and Operations Hours.
- A benchmark was established to determine adequate staffing based on 1 employee having a minimum of 160 hours of productivity per month.

RESULTS

Table 1. Initial Survey Results

Productivity Task	Median Time Spent (minutes)	Productivity Task	Median Time Spent (minutes)
New Prior Authorization	30	Clinical Interventions	30
Prior Authorization Appeal	52.5	340B Contract Pharmacies*	60
Free Drug	30	Orders*	15
Copay Card	15	Teaching/Training*	60
Grant	22.5	Transfers/Referrals*	45
Initial Assessment	60	REMS Management/Drug Information*	30
Reassessment	45	Drug Reps/Community Involvement*	30
Plan of Care	15	Communication*	52.5
Refill Coordination	15	White Bagging*	45
Counseling	15		

*Additional tasks from the second survey that needed to be included in the productivity data.

Table 2. Hierarchy of "Wrap up" Tags

Hierarchy	Median Time Spent (minutes)
Initial Clinical Assessment	60
Initial Clinical Assessment + Plan of Care	75
Initial Clinical Assessment + FA Co-pay	75
Initial Clinical Assessment + Plan of Care + FA Co-pay	90
Prior Authorization Appeal	52.5
Clinical Reassessment	45
Clinical Reassessment + Refill Outreach	60
Clinical Reassessment + Plan of Care	60
Clinical Reassessment + Plan of Care + Refill Outreach	75
Medication Management	37.5
Initial PA Coordination	30
PA Renewal	30
FA New	30
FA Renewal	30
Clinical Intervention	30
FA Grant	22.5
FA Co-pay	15
Plan of Care	15
Refill Outreach	15
Rx Marked as Ready to Fill	5

Table 3. Productivity Results January 2022 – April 2023

Clinical Summary		
Position	Total Hours	
Pharmacist	12,618.5	
Technician	7,507.75	
Pharmacy Student	195.13	
Total	20,321.38	
Operations Summary		
Type	Total Fill Hours	Total Verification Hours
Specialty	786.1	786.04
Retail	699.59	399.7
Total	1,485.69	1,185.74
Shipping Summary		
Total Shipping Hours	4,794.63	

DISCUSSION

- This benchmark and productivity model was created to measure productivity of staff members.
- Not all aspects of time spent doing clinical tasks were able to be accounted for.
- This model was unable to completely account for all the time spent by staff precepting students or attending continuing education and conferences.
- I-Vents have been implemented to try to better capture time spent on clinical interventions and track their outcomes. This will allow specialists to delegate time spent on things not associated with "wrap up" tags but are important for patient care.
- Results from this benchmarking and productivity tool have provided a foundation for evaluation of productivity in clinical, operations, and shipping in a Health System Specialty Pharmacy.

REFERENCES

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